

CLAYBORNE, KAREN D Admin Sex: **Female** DOB: **01/11/1966**

Continuity of Care Document

Summarization of Episode Note | 11/9/2025 to 11/9/2025

Source: Northeast Baptist Hospital

Created: 11/10/2025

Demographics

Contact Information:

19535 CRYSTAL OAK, SAN ANTONIO, TX 78258-3001, USA

19535 CRYSTAL OAK, SAN ANTONIO, TX 78258-3001, USA

Tel: (210)895-0906 (Primary Home)

Tel: (210)895-0906 (Primary Home)

Tel: 2108950906 (Mobile)

Previous Address(es):

--

Marital Status: Single

Religion: Other

Race: Black or African American, Black,

Previous Name(s):

CLAYBORNE, KAREN D

Date of Death: //

Ethnic Group: Not Hispanic or Latino

Language: English

ID: URN:CERNER:IDENTITY-FEDERATION:REALM:8B5D3E6C-9CB3-41B7-8294-6E58968ECFE6-

CH:PRINCIPAL:DR5V727D2MD3EM7V, 200377440

Care Team

Type	Name	Represented Organization	Address	Phone
primary care physician	PCP INFO UNAVAILABLE, UNKNOWN	--	--	--

Related Persons

Relationship	Name	Address	Phone
mother	CLAYMORE THOMAD, JEAN	--	--
	CLAYBORNE, KAREN D	--	--
	CLAYBORNE, KAREN D	--	--

Document Details

Source Contact Info

8811 Village Drive, San Antonio, TX 78217-5415, USA

Tel: (210)297-2000

Author Contact Info

11/10/2025 2:12 AM

Northeast Baptist Hospital

Recipient Contact Info

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Healthcare Professionals

No Data to Display

IDs & Code Type Data

Document Type ID: 2.16.840.1.113883.1.3 : POCD_HD000040

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Document Type Code: 2.16.840.1.113883.6.1, 34133-9

Document Language Code: en-US

Document Set ID: --

Document Version Number: --

Primary Encounter

Encounter Information

Registration Date: 11/9/2025

Discharge Date: 11/9/2025

Visit ID: --

Location Information

Northeast Baptist Hospital

(Work): 8811 Village DriveTeresa Duran, M.D., San Antonio, TX 78217-5440, USA

Providers

Type	Name	Address	Phone
Admitting	KHAN MD, FAHAD	(Work): 111 DALLAS STREET, SAN ANTONIO, TX 78205-	Tel: (330)994-4590 (Work)
Attending	KHAN MD, FAHAD	(Work): 111 DALLAS STREET, SAN ANTONIO, TX 78205-	Tel: (330)994-4590 (Work)
Referring	KHAN MD, FAHAD	(Work): 111 DALLAS STREET, SAN ANTONIO, TX 78205-	Tel: (330)994-4590 (Work)

Encounter

NBH PBAR Account Number 461857716 Date(s): 11/9/25 - 11/9/25

Northeast Baptist Hospital 8811 Village Drive Teresa Duran, M.D. San Antonio, TX 78217-5440 US 210-297-2000

Encounter Diagnosis

Lung cancer metastatic to brain (Discharge Diagnosis) - 11/9/25

Discharge Disposition: Home/Self Care

Attending Physician: KHAN MD, FAHAD

Admitting Physician: KHAN MD, FAHAD

Referring Physician: KHAN MD, FAHAD

Encounter Type: 3 - Emergency

Allergies, Adverse Reactions, Alerts

No Known Allergies

Author: Sutton RN, Nathan, North Central Baptist Hospital

Last Modified: 11/16/2024 10:18 PM

Treatment Plan

No data available for this section

Functional Status

No data available for this section

Immunizations

No data available for this section

Medications

dexamethasone 4 mg oral tablet

4 mg = 1 tab, Oral, Daily, 0 Refill(s)

Start Date: 6/13/25

Status: Ordered

Medication Dispense Status: Completed

Total Allowed Fills: 1

Fills Dispensed: 0

Author: GORMAN FNP, LOREN, Northeast Baptist Hospital

Last Modified: 06/13/2025 8:25 PM

Eliquis 5 mg oral tablet

5 mg = 1 tab, Oral, BID, # 60 tab, 0 Refill(s), 0, Pharmacy: CVS/pharmacy #8325, 158, cm, 06/12/25 6:38:00 CDT, Height, 74.95, kg,

06/10/25 17:42:00 CDT, Dosing Weight

Start Date: 6/13/25

Status: Ordered

Medication Dispense Status: Completed

Quantity: 60.0 Unit: tab

Total Allowed Fills: 1

Fills Dispensed: 0

Author: GORMAN FNP, LOREN, Northeast Baptist Hospital

Last Modified: 06/13/2025 8:25 PM

Eliquis Starter Pack for Treatment of DVT and PE 5 mg oral tablet

= 1 packet(s), Oral, As Directed, # 74 tab, 0 Refill(s), 0, Pharmacy: CVS/pharmacy #8325, 158, cm, 06/12/25 6:38:00 CDT, Height, 74.95, kg, 06/10/25 17:42:00 CDT, Dosing Weight

Start Date: 6/13/25

Stop Date: 7/13/25

Status: Ordered

Medication Dispense Status: Completed

Quantity: 74.0 Unit: tab

Total Allowed Fills: 1

Fills Dispensed: 0

Author: GORMAN FNP, LOREN, Northeast Baptist Hospital

Last Modified: 06/13/2025 8:25 PM

KCl-20 oral liquid

20 mEq = 15 mL, Oral, Daily, # 150 mL, 0 Refill(s), 0, Pharmacy: CVS/pharmacy #8325, 157, cm, 05/30/25 14:39:00 CDT, Height, 83, kg, 05/30/25 14:39:00 CDT, Dosing Weight

Start Date: 5/30/25

Stop Date: 6/9/25

Status: Ordered

Medication Dispense Status: Completed

Quantity: 150.0 Unit: mL

Total Allowed Fills: 1

Fills Dispensed: 0

Author: CROWNOVER JR MD, ROY M, Northeast Baptist Hospital

Last Modified: 05/30/2025 11:20 PM

osimertinib 80 mg oral tablet

Daily, 0 Refill(s)

Start Date: 5/31/25

Status: Ordered

Medication Dispense Status: Completed

Total Allowed Fills: 1

Fills Dispensed: 0

Author: Sehr RN, Vanessa, Northeast Baptist Hospital

Last Modified: 05/31/2025 7:01 AM

pantoprazole 40 mg oral enteric coated tablet

40 mg = 1 tab, Oral, Daily, # 30 tab, 0 Refill(s), 0, Pharmacy: CVS/pharmacy #8325, 158, cm, 06/03/25 3:45:00 CDT, Height, 83.7, kg, 05/31/25 2:09:00 CDT, Dosing Weight

Start Date: 6/3/25

Status: Ordered

Medication Dispense Status: Completed

Quantity: 30.0 Unit: tab

Total Allowed Fills: 1

Fills Dispensed: 0

Author: EAST, SHERALYN, Northeast Baptist Hospital

Last Modified: 06/3/2025 5:06 PM

ReliOn/NovoLIN 70/30 FlexPen subcutaneous suspension

25 Unit(s), Subcut, BID, # 10 mL, 0 Refill(s), 0, Pharmacy: CVS/pharmacy #8325, 158, cm, 06/03/25 3:45:00 CDT, Height, 83.7, kg, 05/31/25 2:09:00 CDT, Dosing Weight

Start Date: 6/3/25

Status: Ordered

Medication Dispense Status: Completed

Quantity: 10.0 Unit: mL

Total Allowed Fills: 1

Fills Dispensed: 0

Author: EAST, SHERALYN, Northeast Baptist Hospital

Last Modified: 06/3/2025 5:06 PM

Mental Status

No data available for this section

Problem List

Condition	Confirmation	Course	Effective Dates	Status	Health Status	Informant
Acute deep vein thrombosis of lower limb<7>Finding<10>2967724010<10>SNOMED CT<10>SNOMED!651000119108<8> ¹ Author: Tenet Use for CCDA Provenance Last Modified: 06/1/2025 1:31 PM	Confirmed			Active Author: Tenet Use for CCDA Provenance Last Modified: 06/1/2025 1:31 PM		
Embolism from thrombosis of vein of lower extremity<7>Finding<10>2871476015<10>SNOMED CT<10>SNOMED!444816006<8> ^{2, 3} Author: Tenet Use for CCDA Provenance Last Modified: 06/3/2025 5:06 PM	Confirmed			Active Author: Tenet Use for CCDA Provenance Last Modified: 06/3/2025 5:06 PM		
Lung cancer Author: Grissett, Talya, Northeast Baptist Hospital Last Modified: 12/26/2024 3:59 PM	Confirmed			Active Author: Grissett, Talya, Northeast Baptist Hospital Last Modified: 12/26/2024 3:59 PM		patient
Pulmonary embolism ⁴ Author: Tenet Use for CCDA Provenance Last Modified: 06/14/2025 1:54 PM	Confirmed			Active Author: Tenet Use for CCDA Provenance Last Modified: 06/14/2025 1:54 PM		

¹ Problem added due to documentation of Acute embolism and thrombosis of unspecified deep veins of unspecified lower extremity by EAST, SHERALYN MLP NP R3 .

Author: Tenet Use for CCDA Provenance

Last Modified: 06/1/2025 1:31 PM

² Problem updated due to documentation of Acute embolism and thrombosis of unspecified deep veins of left lower extremity by EAST, SHERALYN MLP NP R3 .

Author: Tenet Use for CCDA Provenance

Last Modified: 06/3/2025 5:06 PM

³ Problem added due to documentation of Acute embolism and thrombosis of unspecified deep veins of left lower extremity by GARZA MD, ANNAA ED Physician Final ECG R3 .

Author: Tenet Use for CCDA Provenance

Last Modified: 06/3/2025 5:06 PM

⁴ Problem added due to documentation of Other pulmonary embolism without acute cor pulmonale by GORMAN FNP, LOREN MLP NP R3

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Procedures

No data available for this section

Results

Laboratory List

Name	Date
Partial Thromboplastin Time (PTT)	11/9/25
Prothrombin Time (PT)	11/9/25
Complete Blood Count With Auto Differential (CBC w/auto Diff)	11/9/25
Comprehensive Metabolic Panel (CMP)	11/9/25
Magnesium Level	11/9/25

Most recent to oldest [Reference Range]:	1
NRBC Auto Abs [0.00-0.02 x10(3)/mcL]	0.03 x10(3)/mcL *HI* (11/9/25 4:16 PM) Author: Northeast Baptist Hospital Last Modified: 11/9/2025 10:44 PM
Slide Review	To Follow *NA* (11/9/25 4:16 PM) Author: Northeast Baptist Hospital Last Modified: 11/9/2025 10:44 PM
eGFR CKD-EPI [≥ 90 mL/min/1.73m ²]	70 mL/min/1.73m ² ¹ *LOW* (11/9/25 4:16 PM) Author: Northeast Baptist Hospital Last Modified: 11/9/2025 10:47 PM
AGAP	13 *NA* (11/9/25 4:16 PM) Author: Northeast Baptist Hospital Last Modified: 11/9/2025 10:47 PM
Globulin	4 *NA* (11/9/25 4:16 PM) Author: Northeast Baptist Hospital Last Modified: 11/9/2025 10:47 PM
PT [9.4-11.4 sec]	10.9 sec (11/9/25 5:11 PM) Author: Northeast Baptist Hospital Last Modified: 11/9/2025 11:37 PM

Most recent to oldest [Reference Range]:	1
PTT [23.9-32.6 sec]	29.0 sec ² (11/9/25 5:11 PM) <i>Author: Northeast Baptist Hospital</i> <i>Last Modified: 11/9/2025 11:37 PM</i>
RBC [3.63-4.92 x10(6)/mcl]	5.33 x10(6)/mcl *HI* (11/9/25 4:16 PM) <i>Author: Northeast Baptist Hospital</i> <i>Last Modified: 11/9/2025 10:44 PM</i>
Sodium Lvl [136-145 mEq/L]	139 mEq/L (11/9/25 4:16 PM) <i>Author: Northeast Baptist Hospital</i> <i>Last Modified: 11/9/2025 10:47 PM</i>
Total Protein [6.4-8.9 g/dL]	8.4 g/dL (11/9/25 4:16 PM) <i>Author: Northeast Baptist Hospital</i> <i>Last Modified: 11/9/2025 10:47 PM</i>
Albumin Lvl [3.5-5.7 g/dL]	4.1 g/dL (11/9/25 4:16 PM) <i>Author: Northeast Baptist Hospital</i> <i>Last Modified: 11/9/2025 10:47 PM</i>
Alk Phos [34-104 units/L]	95 units/L (11/9/25 4:16 PM) <i>Author: Northeast Baptist Hospital</i> <i>Last Modified: 11/9/2025 10:47 PM</i>
ALT [7-52 units/L]	17 units/L (11/9/25 4:16 PM) <i>Author: Northeast Baptist Hospital</i> <i>Last Modified: 11/9/2025 10:47 PM</i>
AST [13-39 units/L]	15 units/L (11/9/25 4:16 PM) <i>Author: Northeast Baptist Hospital</i> <i>Last Modified: 11/9/2025 10:47 PM</i>
Bili Total [0.3-1.0 mg/dL]	0.3 mg/dL (11/9/25 4:16 PM) <i>Author: Northeast Baptist Hospital</i> <i>Last Modified: 11/9/2025 10:47 PM</i>
CO2 [21-31 mEq/L]	25 mEq/L ³ (11/9/25 4:16 PM) <i>Author: Northeast Baptist Hospital</i> <i>Last Modified: 11/9/2025 10:47 PM</i>
Glucose Level [74-109 mg/dL]	116 mg/dL *HI* (11/9/25 4:16 PM) <i>Author: Northeast Baptist Hospital</i> <i>Last Modified: 11/9/2025 10:47 PM</i>

Most recent to oldest [Reference Range]:	1
Hct [31.2-41.9 %]	46.0 % *HI* (11/9/25 4:16 PM) <i>Author: Northeast Baptist Hospital</i> <i>Last Modified: 11/9/2025 10:44 PM</i>
Hgb [10.9-14.3 g/dL]	15.0 g/dL *HI* (11/9/25 4:16 PM) <i>Author: Northeast Baptist Hospital</i> <i>Last Modified: 11/9/2025 10:44 PM</i>
INR [0.9-1.1 ratio]	1.0 ratio ⁴ (11/9/25 5:11 PM) <i>Author: Northeast Baptist Hospital</i> <i>Last Modified: 11/9/2025 11:37 PM</i>
Magnesium Lvl [1.9-2.7 mg/dL]	2.3 mg/dL (11/9/25 4:16 PM) <i>Author: Northeast Baptist Hospital</i> <i>Last Modified: 11/9/2025 10:47 PM</i>
MCH [24.3-33.2 pg]	28.2 pg (11/9/25 4:16 PM) <i>Author: Northeast Baptist Hospital</i> <i>Last Modified: 11/9/2025 10:44 PM</i>
MCHC [32.5-35.8 g/dL]	32.6 g/dL (11/9/25 4:16 PM) <i>Author: Northeast Baptist Hospital</i> <i>Last Modified: 11/9/2025 10:44 PM</i>
MCV [73.7-95.5 fL]	86.3 fL (11/9/25 4:16 PM) <i>Author: Northeast Baptist Hospital</i> <i>Last Modified: 11/9/2025 10:44 PM</i>
MPV [7.5-11.2 fL]	9.3 fL (11/9/25 4:16 PM) <i>Author: Northeast Baptist Hospital</i> <i>Last Modified: 11/9/2025 10:44 PM</i>
Potassium Lvl [3.4-5.0 mEq/L]	4.0 mEq/L (11/9/25 4:16 PM) <i>Author: Northeast Baptist Hospital</i> <i>Last Modified: 11/9/2025 10:47 PM</i>
WBC [3.6-11.2 x10(3)/mcL]	9.5 x10(3)/mcL (11/9/25 4:16 PM) <i>Author: Northeast Baptist Hospital</i> <i>Last Modified: 11/9/2025 10:44 PM</i>
BUN [7-25 mg/dL]	14 mg/dL (11/9/25 4:16 PM) <i>Author: Northeast Baptist Hospital</i> <i>Last Modified: 11/9/2025 10:47 PM</i>

Most recent to oldest [Reference Range]:	1
Calcium Lvl [8.6-10.3 mg/dL]	9.6 mg/dL (11/9/25 4:16 PM) <i>Author: Northeast Baptist Hospital</i> <i>Last Modified: 11/9/2025 10:47 PM</i>
Lymphocyte Abs [1.0-3.0 x10(3)/mcL]	0.6 x10(3)/mcL *LOW* (11/9/25 4:16 PM) <i>Author: Northeast Baptist Hospital</i> <i>Last Modified: 11/9/2025 10:44 PM</i>
Platelet Count [159-386 x10(3)/mcL]	184 x10(3)/mcL (11/9/25 4:16 PM) <i>Author: Northeast Baptist Hospital</i> <i>Last Modified: 11/9/2025 10:44 PM</i>
Neutrophil Rel [43.3-76.6 %]	87.7 % *HI* (11/9/25 4:16 PM) <i>Author: Northeast Baptist Hospital</i> <i>Last Modified: 11/9/2025 10:44 PM</i>
Lymphocyte Rel [16.0-43.5 %]	6.3 % *LOW* (11/9/25 4:16 PM) <i>Author: Northeast Baptist Hospital</i> <i>Last Modified: 11/9/2025 10:44 PM</i>
Monocyte Rel [4.5-12.5 %]	5.4 % (11/9/25 4:16 PM) <i>Author: Northeast Baptist Hospital</i> <i>Last Modified: 11/9/2025 10:44 PM</i>
Monocyte Abs [0.3-1.0 x10(3)/mcL]	0.5 x10(3)/mcL (11/9/25 4:16 PM) <i>Author: Northeast Baptist Hospital</i> <i>Last Modified: 11/9/2025 10:44 PM</i>
Eosinophil Rel [0.6-7.9 %]	0.3 % *LOW* (11/9/25 4:16 PM) <i>Author: Northeast Baptist Hospital</i> <i>Last Modified: 11/9/2025 10:44 PM</i>
Eosinophil Abs [0.0-0.5 x10(3)/mcL]	0.0 x10(3)/mcL (11/9/25 4:16 PM) <i>Author: Northeast Baptist Hospital</i> <i>Last Modified: 11/9/2025 10:44 PM</i>
Basophil Rel [0.2-1.4 %]	0.3 % (11/9/25 4:16 PM) <i>Author: Northeast Baptist Hospital</i> <i>Last Modified: 11/9/2025 10:44 PM</i>
Basophil Abs [0.0-0.1 x10(3)/mcL]	0.0 x10(3)/mcL (11/9/25 4:16 PM) <i>Author: Northeast Baptist Hospital</i> <i>Last Modified: 11/9/2025 10:44 PM</i>

Most recent to oldest [Reference Range]:	1
Neutrophil Abs [1.8-7.8 x10(3)/mcL]	8.3 x10(3)/mcL *HI* (11/9/25 4:16 PM) <i>Author: Northeast Baptist Hospital</i> <i>Last Modified: 11/9/2025 10:44 PM</i>
Creatinine Lvl [0.60-1.20 mg/dL]	0.94 mg/dL (11/9/25 4:16 PM) <i>Author: Northeast Baptist Hospital</i> <i>Last Modified: 11/9/2025 10:47 PM</i>
Chloride Lvl [98-107 mEq/L]	105 mEq/L (11/9/25 4:16 PM) <i>Author: Northeast Baptist Hospital</i> <i>Last Modified: 11/9/2025 10:47 PM</i>
NRBC Auto Rel [0.0-0.4 /100WBC]	0.3 /100WBC (11/9/25 4:16 PM) <i>Author: Northeast Baptist Hospital</i> <i>Last Modified: 11/9/2025 10:44 PM</i>
RDW-CV [12.3-17.0 %]	15.5 % (11/9/25 4:16 PM) <i>Author: Northeast Baptist Hospital</i> <i>Last Modified: 11/9/2025 10:44 PM</i>

¹ Interpretive Data: The CKD-EPI (2021) equation is recommended by the National Kidney Foundation-American Society of Nephrology (NKF-ASN) Task Force and incorporates serum creatinine, age, and sex, but does not use a race coefficient. The NKF-ASN task force recommends using cystatin C combined with serum creatinine to confirm eGFR in adults who are at risk for or have chronic kidney disease. Results for patients <18 years of age should be interpreted with caution.

Author: Northeast Baptist Hospital

Last Modified: 11/9/2025 10:47 PM

² Interpretive Data: PTT Interpretive Data

The usual therapeutic range for porcine intestinal heparin is 45.5 -62.0 seconds and 0.3-0.7 anti-Xa units when administered by continuous I.V. drip.

New oral anticoagulant agents (direct thrombin inhibitors and direct factor Xa inhibitors) may prolong aPTT; however, prolongation may be non-linear or not directly related to dose. Routine coagulation monitoring is not currently recommended for these medications.

Author: Northeast Baptist Hospital

Last Modified: 11/9/2025 11:37 PM

³ Interpretive Data: High lactate dehydrogenase (LDH) concentrations in patient samples may cause falsely increased bicarbonate results- if markedly elevated LDH levels are suspected please assess results in conjunction with patient's LDH values.

Author: Northeast Baptist Hospital

Last Modified: 11/9/2025 10:47 PM

⁴ Interpretive Data: INR Interpretive Data

The usual therapeutic range for warfarin (Coumadin) - INR = 2.0-3.0 (PT = 19.9 - 29.0 seconds).

Mechanical heart valves may require the higher INRs - see INDICATION.

INDICATION

USUAL INR

- | | |
|--|---------|
| 1. Prophylaxis of venous thromboembolism in high-risk surgery: | 2.0-3.0 |
| 2. Prophylaxis of venous thromboembolism in hip surgery: | 2.0-3.0 |
| 3. Treatment of deep vein thrombosis or pulmonary embolism: | 2.0-3.0 |
| 4. Prevention of systemic embolism in patients with atrial fibrillation, | |

valvular heart disease, tissue heart valves, or acute myocardial infarction:

2.0-3.0

5. Mechanical prosthetic heart valves (high-risk) = 24.5 – 33.5 seconds: 2.5-3.5

New oral anticoagulant agents (direct thrombin inhibitors and direct factor Xa inhibitors) may prolong PT/INR; however, prolongation may be non-linear or not directly related to dose. Routine coagulation monitoring is not currently recommended for these medications.

Author: Northeast Baptist Hospital

Last Modified: 11/9/2025 11:37 PM

Radiology Reports

Exam Date Time	Procedure	Performing Provider	Status
11/9/25 4:22 PM	XR Chest 1 View Portable		Auth (Verified)

Notes:

(XR Chest 1 View Portable) Reason For Exam: Pain

REPORT

XR Chest 1 View Portable

INDICATION: Pain

COMPARISON:6 months prior

Findings:The heart is normal in size. There is right pleural effusion and underlying atelectasis or infectious infiltrate cannot be excluded. There is degenerative disc disease at line dextroscoliosis of thoracic spine. No lytic or blastic changes are seen.

IMPRESSION:

Right pleural effusion cannot exclude underlying atelectasis or infectious infiltrate.

****Final Report****

Dictated By: MITCHELL MD, DAVID E

Dictated: 11/09/2025 16:27

Signed By: MITCHELL MD, DAVID E

Electronic Signature: 11/09/2025 16:28

Author: Montilva RN, Marissa, Northeast Baptist Hospital

Last Modified: 11/10/2025 1:34 AM

Exam Date Time	Procedure	Performing Provider	Status
11/9/25 5:46 PM	CT Head or Brain w/ + w/o Contrast		Auth (Verified)

Notes:

(CT Head or Brain w/ + w/o Contrast) Reason For Exam: hx of stage 4 lung adenocarcinoma, known mets to brain near cerebellum, presenting with complaints of ongoing difficulty walking, on decadron daily;Brain Mets

REPORT

CT Head or Brain WOW Contrast

CLINICAL HISTORY: Female, 59 years old, with history of hx of stage 4 lung adenocarcinoma, known

metastases to brain near cerebellum, presenting with complaints of ongoing difficulty walking, on decadron daily

TECHNIQUE: Axial images were obtained throughout the head before and after the administration of contrast. This study was performed using dose reduction techniques to include automated exposure control and/or adjustment of mA and/or kV according to patient's size.

COMPARISON EXAMS: CT head from May 30, 2025 and MRI brain from June 12, 2025

FINDINGS: The ventricles, sulci, and cisterns are overall appropriate size, shape dictation for age. There is a nonspecific area of nodular enhancement measuring 1 size involving the left frontal lobe with possible minimal adjacent vasogenic edema concerning for metastatic disease given history. This is best seen on postcontrast axial series image 23. Findings are relatively occult on noncontrast series. There is a 6 mm focus of nodular enhancement involving the mid cerebellar vermis posterior to the fourth ventricle on image 43 with probable adjacent vasogenic edema. This likely correlates with the mixed cystic and solid enhancing mass observed on prior MRI from June 12, 2025. Recommend repeat MRI for further evaluation. No additional areas of abnormal intracranial enhancement are appreciated. No acute osseous findings are present. No bone lesions are seen. Visualized sinuses are clear. Middle ears and mastoid air cells are adequately aerated.

IMPRESSION:

1. Nonspecific nodular enhancement involving the left frontal lobe and cerebellar vermis concerning for metastatic disease given history. Left frontal focus is new from prior imaging. Recommend repeat MRI with and without contrast for further evaluation.
2. No evidence of acute intracranial hemorrhage or herniation.

****Final Report****

Dictated By: BRUTON MD, ROBERT H

Dictated: 11/09/2025 17:52

Signed By: BRUTON MD, ROBERT H

Electronic Signature: 11/09/2025 18:02

Author: PERALES NP, JESSICA, Northeast Baptist Hospital

Last Modified: 11/10/2025 12:41 AM

Vital Signs

No data available for this section

Social History

Social History Type	Response
Smoking Status	Former smoker entered on: 11/9/25 <i>Author: Jimenez RN, Damian, Northeast Baptist Hospital</i> <i>Last Modified: 11/9/2025 11:07 PM</i>

Social History Type	Response
Birth Sex <i>Author: Tenet Use for CCDA Provenance</i> <i>Last Modified: 11/10/2025 9:22 AM</i>	
Sex Representation <i>Author: Tenet Use for CCDA Provenance</i> <i>Last Modified: 11/10/2025 9:22 AM</i>	Female (finding)

Health Concerns

No data available for this section

Implantable Device List

No data available for this section

Hospital Discharge Instructions

Section Author: KHAN MD, FAHAD, Northeast Baptist Hospital, 11/10/2025 1:50 AM

Follow Up Care

11/09/2025 13:48:23

With: Follow up with Specialist

Address: Unknown

When: 3-5 days

Comments: your oncologist

With: Return to Emergency Department

Address: Unknown

When: As needed

With: Follow up with Primary Care Physician

Address: Unknown

When: 2-3 days

With: OSAMAAHMED

Address:

12702 IH 35 N

SUITE 101

LIVE OAK, TX 78240-

2106254733 Business (1)

When: 3-5 days

Comments: Neurosurgeon

Goals

No data available for this section

Reason for Referral

No data available for this section

Hospital Course

No data available for this section

Clinical Note

No data available for this section

Patient Care team information

Care Team Personnel

Name: PCP INFO UNAVAILABLE, UNKNOWN

Position: RO No Access

Member Role: Primary Care Physician

Author: Tenet Use for CCD A Provenance

Last Modified: 04/9/2025 4:49 AM

Care Team Related Persons

Name: CLAYMORE THOMAD, JEAN

Author: Tenet Use for CCD A Provenance

Last Modified: 12/4/2024 7:24 PM

Family History

No data available for this section

Insurance Providers

Guarantor name: NA

Health Plan Information #: 1

Payer: SUPERIOR HLTH PLANS

Payer Identifier: NA

Member Number: UZ0003012

Group Number: MP_TX_HIOS

Subscriber Identifier: UZ0003012

Relationship to Subscriber: NA

Coverage Type: NA

Coverage Verification Date: NA

Telecom: NA

Address: NA